



Medication Profile for Community Mental Health Consumers Procedure

1. Scope

Site	Operational Area	Applicable to
Armadale Health Service	Mental Health	Medical, Nursing, Allied Health and Clerical

2. Purpose

Community consumers are required to have a current Patient Medication Profile in their health record to minimise communication errors and to ensure safety with use of medications.

3. Background

Medicines are associated with a higher incidence of errors and adverse events than other healthcare interventions. Some of these events are costly in terms of morbidity, mortality and resources. Many are avoidable. Armadale Kalamunda Group (AKG) is committed to strategies to support safe, medication management.

The intention of the National Safety and Quality Health Service Standard (NSQHS), Standard 4: Medication Safety is to ensure that competent clinicians safely prescribe, dispense and administer appropriate medicines to informed patients and monitor the effects.

4. Procedure

- All medical staff are to ensure that every community mental health consumer who are prescribed medication by the service, have a current Patient Medication Profile within their health record.
 - This includes medications with non- psychiatric indications (e.g. atropine drops to manage hyper salivation secondary to clozapine), as well as non- oral dosage formulations (e.g.: antipsychotic depot).
- For consumers who require staged medication supply (e.g. weekly dispensing) as a risk management strategy, it is important to record such details on the medication profile form (AKMR169), to inform pharmacy and other involved clinical staff.
- The Patient Medication Profile (AKMR169) is to be placed in the front of the Pharmaceutical Benefits Scheme (PBS) section of each health record.
- Whenever a community consumer's medications are revised or amended, it is to be recorded and updated on the Patient Medication Profile Form, Best Practice and emailed to Armadale Health Service: AHS.PharmacyDispensary@health.wa.gov.au with the consumer's prescriptions.
- Consumers and carers are to be provided with a comprehensive verbal explanation of medication treatment options, benefits and associated risks and side-effects, compliance and

discontinuation issues; and offered appropriate written pharmaceutical information, when commencing a new medication. If appropriate the use of an interpreter to convey important medication options should be used.

- A summary of the conversation and the provision of medication information are to be documented in the consumer's health record via the medication sticker.
- PSOLIS alerts are to be used to identify consumers at risk of misusing medications and be included in the clinical review form and management plan

5. Legislative context

The following documents are required to give effect to this procedure [i.e. the documents included are mandatory]:

- *WA Mental Act 2014*
- *WA Pharmacy Act 2010*
- *WA Medicines and Poisons Act 2014*
- HDWA [Mental Health Consumer Medication Information Policy \(MP 0069/17\)](#)

6. Supporting information

The following documents support the implementation of this procedure:

- [Consumer Medication Information and Resources](#)
- HDWA [Mental Health conditions, treatment and medications \(printable leaflets\)](#)
- HDWA [Mental Health Charts and Clozapine Resources](#)
- [LUNSERS \(Liverpool University Neuroleptic Side Effect Rating Scale\) – Using for Monitoring Neuroleptic Side Effect Guideline \(MEN-GUI-0230-11\)](#)
- [Medication Management Procedure \(AKG-PRO-0255-18\)](#)
- [Best Practice Prescription Program](#)

7. Compliance, monitoring and evaluation

Compliance against this procedure will be monitored by the Mental Health Safety and Quality Departmental meeting.

Monitoring activities will include:

- Mental Health documentation audits.
- Medication Sticker Audit

8. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- Standard 2 – Action 4.3 Partnering with consumers

Clinicians use organisational processes from the Partnering with Consumers Standard in medication management to:

- a. Actively involve patients in their own care
- b. Meet the patient's information needs
- c. Share decision-making

- Standard 4 – Action 4.7 Adverse drug reactions

The health service organisation has processes for documenting a patient's history of medicine allergies and adverse drug reactions in the healthcare record on presentation

- Standard 4 – Action 4.10 Medication review

The health service organisation has processes:

- a. To perform medication reviews for patients, in line with evidence and best practice
- b. To prioritise medication reviews, based on a patient's clinical needs and minimising the risk of medication-related problems
- c. That specify the requirements for documentation of medication reviews, including actions taken as a result

- Standard 4 – Action 4.11 Information for patients

The health service organisation has processes to support clinicians to provide patients with information about their individual medicines needs and risks

- Standard 4 – Action 4.15 High-risk medicines

The health service organisation:

- a. Identifies high-risk medicines used within the organisation
- b. Has a system to store, prescribe, dispense and administer high-risk medicines safely

National Standards for Mental Health Services (2010):

- Criteria 2.4 The MHS minimises the occurrence of adverse medication events within all MHS settings.
- Criteria 10.5.8 The views of the consumer and their carer(s), and the history of previous treatment is considered and documented prior to administration of new medication and/or other technologies.

Chief Psychiatrist's Standards for Clinical Care

- Standard: Care Planning

9. Policy owner

Shannon Stuart – Program Manager Community Mental Health

Enquiries relating to this procedure may be directed to:

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10. Review

This procedure will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
Version 4	18/05/2023	18/05/2026	Scheduled review

11. Authorisation

This procedure has been authorised and issued by the Service Director Mental Health.

Authorised by	Karen Elliott - Service Director Mental Health
Authorisation date	18/05/2023
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